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PERSONALITY DEVELOPMENT FOLLOWING SEXUAL ABUSE

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Abstract—Thirty-seven girls and twelve boys who had been sexually abused at an average of 2.6 years previously were traced and reviewed. The nonoffending parents participated in a structured interview and the children were assessed using the Piers-Harris Self-Concept Scale and the Achenbach Child Behavior Checklist. Each child was matched with a child not known to have been sexually abused and these control children underwent a similar psychological assessment. Interviews with the nonoffending parents found that 76% of the children were thought to be less confident than before, 30% had fewer friends, and 20% were more aggressive. Increased sexual awareness was noted in 24%. School teachers reported that 28% still had behavior problems, 17% had repeated a year at school, and a further 17% had deteriorated in their school work. The sexually abused girls had significantly lower self-esteem than the control girls. There was no difference in self-esteem between the control and the sexually abused boys. There was a higher incidence of stranger assault in the boys compared with the girls. The Child Behavior Checklist completed by the parents, the Teacher Report Form, and the Youth Self-Report of the Child Behavior Checklist showed that a highly significant proportion of sexually abused children fell into the clinical range. Sexual abuse appears to have long-term adverse consequences for many of the victims. This may have implications for their ability to relate to others, for the adult friendships they will make, and eventually for the way they will relate to their own children.

Résumé—Les auteurs ont retrouvé la trace de 37 filles et 12 garçons qui avaient été victimes de sévices sexuels en moyenne 2.6 années auparavant, et les ont réévalués. La maltraitance n'avait pas été commise par leurs parents. Ceux-ci ont pris part à un interview structuré et leurs enfants ont été testés à l'aide de l'échelle Piers-Harris "self-concept" et à l'aide de la liste sur le comportement de l'enfant développé par Achenbach. On a apparié chaque enfant maltraité avec un enfant n'ayant pas été victime de sévices sexuels et ces enfants témoins ont subi une évaluation psychologique semblable à celles des enfants autrefois maltraités. Les interviews avec les enfants ont démontré que le 76% de ceux-ci avait perdu jusqu'à un certain degré leur confiance en eux-mêmes, le 30% avait moins d'amis qu'auparavant et le 20% était plus agressif. Une conscience plus grande des problèmes sexuels a été rencontrée dans le 24% des enfants. D'après les maîtres d'écoles le 28% des enfants avait encore des problèmes de comportement, le 17% avait dû répéter une année d'école et un autre 17% avait baissé dans le rendement scolaire. Les filles victimes de sévices sexuels avaient une moins bonne opinion d'elles-mêmes que les filles témoins. Chez les garçons par contre, cette différence n'existait pas. Chez les garçons on notait que les sévices avaient plus souvent été infligés par un individu inconnu de la victime que chez les filles. Les listes de contrôle sur le comportement enfantin remplies par les parents et les carnets scolaires rédigés par les maîtres ainsi que les questionnaires sur le comportement remplis par les enfants eux-mêmes ont démontré qu'une proportion hautement significative des enfants victimes de maltraitance sexuelle pouvait être rangé dans le cadre de manifestations que l'on qualifie de cliniques. Un grand nombre des victimes semble souffrir de conséquences à long terme des sévices subis. Ceci peut avoir des implications en ce qui concerne leurs capacités d'établir des relations avec autrui, en ce qui concerne les amitiés qu'ils feront ensuite à l'âge adulte et finalement en ce qui concerne les conséquences sur la façon dont ils se comporteront plus tard à l'égard de leurs propres enfants.

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INTRODUCTION

ALTHOUGH SEXUAL ABUSE of children was first documented by Tardieu in 1860 [1], it was not until Kempe's 1978 work [2] that it became widely recognized by the medical profession. This recognition has been accompanied by increases in both reporting and estimates of the prevalence of sexual abuse [3]. Despite increased professional interest, there is a relative dearth of empirical data assessing the impact of sexual trauma on a child's psychosocial development.

The literature indicates that researchers hold conflicting views on the outcome of child sexual abuse. Some have suggested that sexual abuse has only minimal effects upon children's psychological development [4] and that its traumatic impact has been greatly overstated [5-7], while some believe that there are no particular detrimental effects or subsequent disturbances [8, 9]. Conversely, others report difficulties in sexual adjustment, interpersonal problems, educational difficulties and various psychological symptoms [10], including acute anxiety neuroses, [11] self-destructive acts and somatic symptoms [12], loss of self-esteem [13], prostitution and delinquent criminal behavior [14], depression and actual or attempted suicide [15].

A review of the empirical data shows a paucity of methodologically sound studies supporting these conclusions. One limitation involves the use of disparate definitions of what constitutes child sexual abuse. Some researchers include both contact and noncontact abuse in their definition but report separate prevalence rates for both categories [16]; others limit their studies to victims of contact abuse, but report figures for both contact and noncontact abuse [17]; others combine contact and noncontact groups and report global findings [18, 19]. Some researchers include peer-group perpetrators [16, 17], while others exclude them [18, 19], and there are variations in the ceiling age of abuse, which is 17 years in some studies [16, 17], and 16 years in others [18, 19].

Another methodological problem is in sampling technique. As much of the data has been collected by retrospective interviewing of adults, usually females, from clinical or deviant populations, e.g., psychiatric patients, prisoners and prostitutes [20, 21], information derived from these sources cannot reasonably be extrapolated to represent the population at large. The cause and effect relationship is difficult to establish longitudinally. As sexual abuse usually occurs in the presence of multiple problems, attributing the results to sexual trauma is difficult when they might be simply confounded by the patient or deviant status of the samples in question.

Methodological limitations also mar the conclusions in nonclinical samples. Large numbers of college undergraduates have been surveyed to determine the incidence and outcome of child sexual abuse but such groups are not representative of the general population in terms of social class, ethnicity, and intelligence [22]. Other studies using nonclinical samples provided traveling expenses and child care costs, which introduced a personal motivation bias [23]. The nonclinical samples also required retrospective interviewing techniques, despite the general understanding that memories fade with the passage of time. Most of the studies involved female populations, and, notwithstanding the higher ratio of females to males in sexually abused populations, males have not been fully represented in the literature. Many of the studies lack adequate control or comparison groups [24, 25], and few have utilized standardized psychometric instruments, the data having been drawn from clinical case studies, unstructured psychiatric interviews, law enforcement agencies, court reports, and hospital and welfare files.

The aim of this study is to clarify some of the conflicting information about the outcome of child sexual abuse by reviewing a group of children using a structured interview, standardized psychometric measures, and a matched control group of children who are not known to have been sexually abused.

METHOD

Between January 1981 and December 1983, 229 children (172 girls; 57 boys) were referred to the Sexual Assault Center at the Camperdown Children's Hospital. The mean age at the time of abuse was 8.9 years (range 3-16 years). Of this group 37% were victims of stranger abuse, 32% were abused by relatives, and 31% were abused by acquaintances.

In February 1985, the nonoffending parents or guardians of these children were contacted by mail and offered a review appointment for their child. The mean interval between the sexual abuse and review was 2.6 years. All parents who participated in the study had the aims of the review explained in writing and signed a form consenting to participate.

For each child reviewed, a control child who was not known to have been sexually abused was selected. The school-aged study group children were compared with controls from the same school or school area matched for age (within one week of birthday), sex, socioeconomic status [26], and ethnicity. Postschool-aged study group children were similarly compared with controls matched for age, sex, occupation, socioeconomic status, and ethnicity.

The children and families were assessed with the following measures:

1. Structured Interview. A Questionnaire incorporating a structured interview was designed to record biographical and demographic information, effects of the sexual abuse on the child's behavior at home and at school and details of treatment interventions. The questionnaire was piloted and revised and subsequently administered to the parents or parent-surrogates in the study group.
2. The Piers-Harris Self-Concept Scale [27]. This measure, suitable for children aged 8-18 years, is an 80-item inventory presented in a self-report format, requiring yes/no responses to short, positive and negative statements such as "I like being the way I am"; "My family is disappointed in me." The test yields a total score and six factorially determined subscales of "cluster" scores [28], for behavior, intellectual and school status, physical appearance and attributes, anxiety, popularity, happiness and satisfaction. The scale is widely used in research and clinical settings.
3. The Child Behavior Checklist, revised 1984 profile (CBCL) [29]. This records in a standardized format a checklist of childhood behavior problems and social competencies of children aged 4-16, as reported by their parents or parent figures. This instrument was selected to provide a global assessment of the children's psychosocial adjustment. Pertinent to the present study, it contains several specific questions relating to sexual behavior. An integrated and comprehensive view of the children's behavior was obtained by administering two supplementary checklists. These are the Teacher's Report Form (TRF) [29], using the Achenbach 1985 profile, which is completed by the class teacher; and the Youth Self-Report (YSR) [29], using the Achenbach 1986 profile, which is completed by the children aged 11-18. The responses on the CBCL, TRF and YSR were computer scored using Apple IIe DOS, 3.3 Software.

The study group families were seen for two review sessions. In the first the structured interview was used with the parents. Those whose children were in the 4-16 years age range completed the Child Behavior Checklist. During the second session those children aged between 8-18 completed the Piers-Harris Self-Concept Scale with the Youth Self-Report being completed by those within the 11-18 years age range. With the parents' consent, the teachers of all school-age children were contacted and asked to complete the Teacher's Report Form. The teachers were not aware in most cases that the child had been a sexual abuse victim and were not aware of the reason for this study.

School-aged control group children aged between 8-18 years completed the Piers-Harris Self-Concept Scale; those aged between 11-18 completed the Youth Self-Report; teachers completed the Teacher's Report Form, and parents were contacted and asked to complete the Child Behavior Checklist. Control group children who had left school were asked to complete the Piers-Harris Self-Concept Scale and the Youth Self-Report, and for those still living at home, parents were asked to complete the Child Behavior Checklist. All tests were administered individually by an experienced clinical psychologist.

RESULTS

Forty-nine families agreed to participate in the study. The mean age of their children at the time of review was 11.5 years (range 5-19 years). There were 37 girls and 12 boys.

To see whether bias was introduced because of the nonrespondents, the 180 families who were not seen were compared with the 49 who were seen using demographic variables known about both groups. These were age and sex of the child, social class of the child's family, and relationship of the offender to the child. There were no significant differences between the cases reviewed (study group) and those not reviewed (absent group) on any of these variables (Table 1).

Girls were more likely to have been assaulted by a relative (35%) or acquaintance (43%) than boys, for whom the perpetrator was more often a stranger (58%) (Table 2).

Data obtained from the structured interview questionnaire and the psychological measures were analyzed by the University of Sydney Sample Survey Center using the Statistical Package for the Social Sciences [30].

The Structured Interview Questionnaire

This questionnaire was administered to 46 of the 49 study group parents. Three questionnaires were not completed due to geographical (interstate and intrastate) relocation by respondents.

The interview with the parents showed that 76% of children had reduced confidence following the disclosure of sexual abuse, 30% had fewer friends, 28% played with their friends less often, 24% were thought by their parents to be more sexually aware, and 20%

Table 1. Comparison of Sexually Abused Children Reviewed With Those Not Reviewed on Known Demographic Variables

	Comparison on Age				Comparison on Social Class		
Age	Study Group	Absent Group	Total	Social Class	Study Group	Absent Group	Total
0–3	3	13	16	1	14	60	74
4–7	15	39	54	2	11	53	64
8–11	17	56	73	3	19	50	69
12–19	14	72	86	4	5	17	22
	$n = 49$	$n = 180$	$n = 229$		$n = 49$	$n = 180$	$n = 229$
	$\chi^2 = 0.3964$ $df = 3$ $p > 0.25$ not significant				$\chi^2 = 0.3385$ $df = 3$ $p > 0.25$ not significant		
	Comparison of Sex				Comparison of Offender		
Sex	Study Group	Absent Group	Total	Offender	Study Group	Absent Group	Total
Girls	37	135	172	Stranger	11	73	84
				Acquaint	23	49	72
Boys	12	45	57	Relative	15	58	73
	$n = 49$	$n = 180$	$n = 229$		$n = 49$	$n = 180$	$n = 229$
	$\chi^2 = 0.0127$ $df = 1$ $p > 0.25$ not significant				$\chi^2 = 3.1651$ $df = 2$ $p > 0.25$ not significant		

Table 2. Relationship of Perpetrator to Child

	Girls $N = 37$	Boys $N = 12$
Relative	13 (35%)	4 (33%)
Acquaintance	16 (43%)	1 (8%)
Stranger	8 (21%)	7 (58%)

were reported as being more aggressive. Other problems included withdrawal (4%), concerns about becoming homosexual (4%), anti-male feelings (4%), and a subsequent heavy drinking problem in one older child (See Table 3). Problems were also noted at school soon after the episode of sexual abuse. School work deteriorated in 30% while other behavior problems including aggression, truancy and attention-seeking behavior were common (Table 3).

Psychological Measures

The Piers-Harris Self-Concept Scale. Of the 49 study group children, 45 were in the appropriate age range (8-18 years) for this scale. Average scores are usually considered to be between 31st and 70th percentile (or raw scores between 46 and 60). High scores suggest high self-concept with the low scores suggesting the reverse.

The children who had been sexually abused had significantly lower scores than the controls. The total raw score mean for the study group was 52.8. This fell towards the lower limits of the normal range, while the control group mean of 61.6 was just above the normal range (Table 4).

Raw scores of 13 children in the study group were below 46 (the bottom of the average range) compared with only five control children having scores as low as this. In contrast, 28 of the controls had raw scores above 60, indicating high self-esteem, compared with only 14 of the 45 children who had been sexually abused (Table 5).

Table 3. Reported Effects of Sexual Abuse on Behaviour at Home and at School

Problem	Number $N = 46$	%
At home		
Confidence reduced severely	16	35%
Confidence reduced to a lesser degree	19	41%
Has fewer friends	14	30%
Plays with friends less than before	13	28%
More aggressive	9	20%
Increased sexual awareness	11	24%
Withdrawn	2	4%
Anti-male	2	4%
Worries about becoming homosexual	2	4%
Heavy drinking	1	2%
At school		
School work deteriorated	15	32%
Aggressive	7	15%
Attention-seeking	7	15%
Affection-seeking	2	4%
Behaviour problems	13	28%
Has had to repeat a year	8	17%
Poor work	8	17%
Truancy	3	7%

Table 4. Piers–Harris Self-Concept Scale Means and Standard Deviations for Study and Control Groups on Two-Tailed T-tests

Score	Study Group $N = 45$	Control Group $N = 45$	Significance
Raw score	52.8 (*SD* 13.6)	61.6 (*SD* 11.4)	$p < 0.001$
Percentile score	51.7 (*SD* 29.8)	71.5 (*SD* 24.8)	$p < 0.001$
Stanine score	5.0 (*SD* 2.0)	6.4 (*SD* 1.8)	$p < 0.001$
T-score	51.0 (*SD* 10.1)	57.8 (*SD* 9.5)	$p < 0.001$

Analysis of the cluster scores for each of the six dimensions of the Piers-Harris Self-Concept Scale (behavior, intellectual and school status, physical appearance and attributes, anxiety, popularity, happiness and satisfaction) confirmed this result with the sexually abused children having significantly lower scores than the controls on each of these dimensions (Table 6).

To see whether self-concept differences were more marked in sexually abused boys than in sexually abused girls, self-concept between abused and control boys and between abused and control girls was compared. There was no significant difference in self-concept between abused and control boys, but highly significant differences between the abused and control girls, the abused girls having much lower self-concept than the controls (Table 7). Thus the significant differences between the combined boys and girls Total Raw Score Means and Cluster Scale Means were due to the magnitude of low self-ratings by girls who, as previously indicated, were more likely to have been assaulted by a relative or acquaintance compared with the boys for whom stranger assault was more common.

The Child Behavior Checklist (CBCL). The Child Behavior Checklist was administered to 39 parents whose child had been sexually abused and to 39 control parents. On this scale, each child is assessed on two dimensions—social competence and behavior problems. The latter is scored along two broad band groupings which correspond to the distinction between fearful, inhibited, over-controlled behavior (anxiety, depression, somatic concerns called internalizing) and aggressive, anti-social undercontrolled behavior (hyperactivity, cruelty called externalizing). Scores falling outside normal limits are described as being within the "clinical range."

Teacher Report Form. The Teacher Report Form (TRF) was administered to 28 study group and 28 control group teachers. Each child was assessed on two dimensions: adap-

Table 5. Piers–Harris Self Concept Scale Distribution of Raw Scores in 45 Sexually Abused Children and 45 Controls

	Raw Scores Below 46*	Raw Scores Above 60*
Control group $N = 45$	5	28
Study group $N = 45$	13	14

* Average scores are usually considered to be within the 46–60 Raw Score Range.

Table 6. Percentile Means for Cluster Scores on the Piers–Harris Self-Concept Scale

Cluster Scales	Study Group $N = 45$	Control Group $N = 45$	Significance
Behaviour	46.2 (*SD* 27.9)	59.3 (*SD* 29.9)	$p < 0.025$
Intellectual & School status	41.9 (*SD* 29.1)	54.7 (*SD* 30.2)	$p < 0.04$
Physical appearance and attributes	45.9 (*SD* 33.2)	58.8 (*SD* 29.0)	$p < 0.03$
Anxiety (High score indicates low anxiety)	47.6 (*SD* 32.5)	65.5 (*SD* 25.9)	$p < 0.003$
Popularity	45.6 (*SD* 29.5)	61.5 (*SD* 28.0)	$p < 0.01$
Happiness and satisfaction	47.8 (*SD* 32.7)	61.6 (*SD* 24.7)	$p < 0.015$

tive functioning (i.e., school performance, working hard, appropriate behavior, learning, and happiness) and behavioral problems.

The Youth Self-Report. The Youth Self-Report (YSR) was administered to 30 sexually abused and 30 control children. Each child completed self-ratings on two dimensions, i.e., competence (such as degree of participation and skill in sports, number of jobs and quality of work, number of friends and frequency of contact) and behavior problems. The "clinical range" criteria for the TRF and YSR are identical to those used for the CBCL.

Analyses

Initial analyses of the Child Behavior Checklist indicated a highly significant proportion of study group children in the clinical range, when compared with the control group children on both the parent and teacher reports. Of the 39 CBCLs completed, 64% of the study group children scored in the clinical range compared with 3% in the control group. Of the 28 TRFs completed, 36% of the study group children scored in the clinical ranged compared with 3% in the control group. In contrast, the difference was much less marked in the Youth Self-Report. Of the 30 YSRs completed, 13% of the study group children scored in the clinical range compared with 10% in the control group.

The scores of 15 children, who were of the appropriate age for all three versions of the checklist to be completed are presented in Table 8. This shows that in this group, only the sexually abused children had scores in the clinical range, with the difference being most marked in the parent report where 12 of the 15 children had abnormal profiles.

Table 7. Piers–Harris Self Concept Scale Comparison of Sex Differences Across Groups using Two-tailed T-tests

	N	Raw Score $\bar{x}$	Percentiles $\bar{x}$	Stanines $\bar{x}$	T-Score $\bar{x}$
Study girls	33	51.0303	47.6061	4.8788	49.7879
SD		14.081	30.726	2.103	10.493
Control girls	33	61.7576	71.8788	6.4545	58.3030
SD		12.590	26.855	1.938	10.555
Significance		$p < 0.001$*	$p < 0.001$*	$p < 0.001$*	$p < 0.001$*
Study boys	12	57.8333	62.9167	5.6667	54.2500
SD		11.472	24.869	1.670	8.740
Control boys	12	61.0833	70.4167	1.1667	56.5833
SD		7.549	18.686	1.193	6.317
Significance		.497	.486	.504	.541

* Significant at $p < 0.05$ level on paired T-Test.

Table 8. Children Who Scored in the "Clinical Range" on Each Scale of the Child Behaviour Checklist

Achenbach Checklist	Study Group	Control Group
(CBCL) Parent report $N = 15$	12 (80%)	0
(TRF) Teacher report $N = 15$	5 (33.3%)	0
(YSR) Self report $N = 15$	2 (13.3%)	0

Paired T-tests were performed to examine contrasts between means of study and control groups on each separate version of the Child Behavior Checklist. This showed that on the parent report form of the Child Behavior Checklist, study group means were significantly lower than control group means on all dimensions of social competence (Table 9). On behavior problem criteria, the means for the children who had been sexually abused were significantly elevated on the broad band factors of internalizing and externalizing, and on the separate subscales of aggression, delinquency and somatic complaints and delinquency (Table 10).

Analysis of the Teacher's Report Form showed that the sexually abused children scored significantly lower than the control children on all dimensions of the Adaptive Functioning Scale (Table 11). The behavior problem criteria, from the Teacher's Report Form, showed a significant elevation in the study group on eight subscales: internalizing and externalizing, anxiety, social withdrawal, unpopularity, self-destruction, inattentiveness and aggression (Table 12).

On the competence scales of the Youth Self-Report, the children who had been sexually abused rated themselves significantly lower than did the control children on the activity and social dimension (Table 13). On the behavior problems subscales of the Youth Self-Report, there were no significant differences between the abused and control children on six of of the eight subscales (externalizing, somatic complaints, depression, thought disorder, aggression, and delinquency); but there were significant differences on the internalizing and unpopularity subscales, the abused children rating themselves as internalizing their feelings more and as being less popular than the control children (Table 14).

The significant elevation of behavior problem scores in the study group on the Child Behavior Checklist Parents' Report and on the Teacher's Report Form appears to be related to the high behavior problem profiles of the girls, rather than the boys. By contrast, the elevation of behavior problem scores in the study group on the Youth Self-Report was general and not specifically attributable to girls or boys.

Table 9. Social Competence Means on the Child Behaviour Checklist (Parent Report)

Dimension	Study Group $N = 39$	Control Group $N = 39$	Significance
Activities	40.3 (*SD* 11.3)	50.0 (*SD* 6.0)	$p < 0.001$
Social	40.6 (*SD* 11.2)	51.0 (*SD* 8.0)	$p < 0.001$
School	36.0 (*SD* 10.7)	49.0 (*SD* 6.9)	$p < 0.001$

Table 10. Behaviour Problem Means on the Child Behaviour Checklist (Parent Report)

Problem Subscale	Study Group $N = 39$	Control Group $N = 39$	Significance
Internalising	63.7 (*SD* 10.2)	49.6 (*SD* 6.1)	$p < .001$
Externalising	62.2 (*SD* 10.7)	48.5 (*SD* 7.3)	$p < .001$
Aggressive	64.2 (*SD* 9.1)	55.5 (*SD* 1.3)	$p < .001$
Delinquent	64.6 (*SD* 8.7)	56.8 (*SD* 3.2)	$p < .001$
Somatic complaints	64.5 (*SD* 10.1)	58.1 (*SD* 4.4)	$p < .001$

Table 11. Teacher's Report Form Adaptive Functioning Means

Dimension	Study Group $N = 28$	Control Group $N = 28$	Significance
School performance	45.6 (*SD* 8.1)	60.8 (*SD* 8.7)	$p < .001$
Working hard	45.8 (*SD* 8.1)	55.1 (*SD* 9.8)	$p < .001$
Appropriate behaviour	45.9 (*SD* 10.1)	51.8 (*SD* 9.4)	$p < 0.05$
Learning	45.2 (*SD* 9.1)	53.9 (*SD* 8.2)	$p < 0.05$
Happy	46.8 (*SD* 13.3)	55.2 (*SD* 9.7)	$p < 0.05$

Table 12. Teacher's Report Form Behaviour Problem Means

Problem Subscale	Study Group $N = 28$	Control Group $N = 28$	Significance
Internalising	58.5 (*SD* 8.9)	50.5 (*SD* 6.3)	$p < 0.001$
Externalising	56.2 (*SD* 7.9)	50.1 (*SD* 5.0)	$p < 0.01$
Anxious	61.2 (*SD* 7.1)	56.2 (*SD* 2.3)	$p < 0.01$
Social withdrawal	59.9 (*SD* 5.1)	56.2 (*SD* 3.3)	$p < 0.01$
Unpopular	60.3 (*SD* 4.8)	57.6 (*SD* 3.2)	$p < 0.05$
Self destructive	62.8 (*SD* 6.6)	59.3 (*SD* 3.0)	$p < 0.05$
Inattentive	59.5 (*SD* 6.5)	55.3 (*SD* 1.7)	$p < 0.05$
Aggressive	58.5 (*SD* 4.5)	55.8 (*SD* 2.2)	$p < 0.05$

Table 13. Youth Self Report Competence Means

Dimension	Study Group $N = 30$	Control Group $N = 30$	Significance
Activities	50.0 (*SD* 6.7)	52.9 (*SD* 2.5)	$p < 0.05$
Social	43.8 (*SD* 12.1)	49.4 (*SD* 9.5)	$p < 0.05$

Table 14. Youth Self Report Behaviour Problem Means

Behaviour Problem Subscale	Study Group $N = 30$	Control Group $N = 30$	Significance
Internalising	54.7 (*SD* 9.7)	48.7 (*SD* 11.5)	$p < 0.05$
Externalising	53.3 (*SD* 10.5)	52.2 (*SD* 10.5)	n.s.
Somatic complaints	57.4 (*SD* 3.5)	58.3 (*SD* 4.9)	n.s.
Depressed	59.1 (*SD* 5.6)	56.1 (*SD* 5.1)	n.s.
Unpopular	59.1 (*SD* 6.9)	56.1 (*SD* 2.9)	$p < 0.05$
Thought-disorder	60.8 (*SD* 7.0)	62.4 (*SD* 10.4)	n.s.
Aggressive	57.9 (*SD* 7.4)	57.0 (*SD* 4.2)	n.s.
Delinquent	59.6 (*SD* 7.2)	59.5 (*SD* 6.8)	n.s.

DISCUSSION

This study has reviewed a group of sexually abused children at an average of 2.6 years after the sexual abuse and has compared their personality with a group of matched controls. The abused children have a high incidence of reduced confidence, with aggressive behavior, difficulty with friendships and poor school performance. This has been noted by the parents since the episode of abuse. When compared with control children, the sexually abused children were lower in self-esteem and had a significantly greater number of personality profiles in the clinical range on the Achenbach Child Behavior Checklist, Teacher's Report Form and Youth Self-Report.

Low self-esteem has been noted previously in controlled studies of physically abused children [31, 32], and in uncontrolled studies of sexually abused children [20, 33]. The fact that it has been shown in this study to be significantly more common in sexually abused children than in the control children lends support to the clinical impressions of diminished self-esteem noted by others [35-37].

Of interest is the finding that when sex differences are analyzed, low self-esteem and problematic behaviors were more apparent in the girls. However, when examining the relationship of perpetrator to the child in this study, for the boys the offender was more likely to be a stranger, as occurred in 58% of the cases, than for the girls where 21% had been victims of stranger assault. This may be the reason why self-esteem and behavior problem differences were more obvious in the girls in this study, suggesting the more serious nature of sexual abuse when caused by a relative or acquaintance whom the child knows and trusts. This high incidence of stranger assault in boys is in contrast with the findings of Spencer and Dunklee [37], who report that boys tend to be abused more frequently by relatives (49%) or acquaintances (34%) than by strangers (12%).

The wide discrepancy between the number of clinical range profiles reported for the sexually abused group by parents, teachers, and children themselves deserves mention. Of the 15 children who were in the age range for the completion of all 3 versions of the Achenbach-Edelbrock Checklist, parents reported 12 profiles in the clinical range (80%); children reported 2 profiles in the clinical range (13%); and teachers reported 5 profiles in the clinical range (33%). The low correspondence between parent and child ratings is well reported in the literature [38-43]. Some researchers have shown that children can be accurate and reliable reporters [40, 44, 45] and that parents are biased by their own psycholog-

ical distress [44] and more severe in their ratings [40]. Others say that children underestimate their problems [41] and fail to report symptoms [46]. As a number of the parents of children in the present study had been abused in childhood, their own feelings about sexual abuse may have confounded their interpretation of the checklist items, which has resulted in elevated behavior problem scores for their children. Conversely, the relatively low number of clinical range ratings ascribed to the children by themselves may indicate that the psychological defense mechanisms of repression and denial are operating against the trauma of sexual abuse. The accuracy of either the parent's or the child's reports is difficult to evaluate without a criterion that is separate from the perspective of both parents and children. To some degree, the Teacher's Report Form provided an objective assessment of the children's behavior, for in most instances assessments were "blind" as the teachers were unaware of the episodes of sexual abuse and were not told the reason for this study.

IMPLICATIONS

Although the follow-up period of this study is relatively short (average of 2.6 years), this study does suggest that this group of children have persisting problems with their behavior, the way they are seen by others, and the way they feel about themselves. This has implications for their ability to relate to others, for the adult friendships they will make, and eventually the way they will relate to their own children. If this is true, it means that children who have been sexually abused need a long-term treatment approach if these adverse effects are to be ameliorated and if they are to be given some of the skills which will help them establish satisfactory relationships in their adult lives. We would note that, according to the measures we used, not all of the children we studied were disturbed. This is not to suggest that these children experienced no adverse effects; but it does suggest that some children are more resilient than others and that some forms of sexual abuse, such as stranger compared with relative, persisting abuse compared with single episode abuse, and abuse associated with other family pathology, may have different significance for and different effects on the child.

As well as the need for further controlled studies on the long-term effects of child sexual abuse, research also should be directed to studying which families and which children are most likely to have significant disturbance so that resources, which are relatively scarce in relation to the size of the problem, can be directed more appropriately.

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